HIGHLIGHTS OF PRESCRIBING INFORMATION: METFORMIN HYDROCHLORIDE TABLETS

WARNING: LACTIC ACIDOSIS

Post-marketing cases of metformin-associated lactic acidosis have resulted in de
ath, hypothermia, hypotension, and resistant bradyarrhythmias. The onset of metf
ormin-associated lactic acidosis is often subtle, accompanied only by nonspecifi
c symptoms such as malaise, myalgias, respiratory distress, somnolence, and abdo
minal pain. Risk factors include renal impairment, concomitant use of certain dr
ugs (e.g., carbonic anhydrase inhibitors), age 65 years or older, radiological s
tudy with contrast, surgery and other procedures, hypoxic states, and excessive
alcohol intake. If lactic acidosis is suspected, immediately discontinue metform
in and institute general supportive measures in a hospital setting.

1 INDICATIONS AND USAGE

Metformin hydrochloride is a biguanide indicated as an adjunct to diet and exerc
ise to improve glycemic control in adults and pediatric patients aged 10 years a
nd older with type 2 diabetes mellitus.

2 DOSAGE AND ADMINISTRATION

- Adults: Recommended starting dose is 500 mg orally twice daily or 850 mg once
daily with meals. Increase in increments of 500 mg weekly or 850 mg every 2 week
s up to maximum dose of 2550 mg per day in divided doses.
- Renal Impairment: Assess renal function prior to initiation:
  - eGFR 45 to 59 mL/min/1.73 m2: Initiation is not recommended, but if already
taking, monitor renal function every 3 to 6 months.
  - eGFR 30 to 44 mL/min/1.73 m2: Maximum recommended dose is 1000 mg per day.
  - eGFR < 30 mL/min/1.73 m2: CONTRAINDICATED.
- Discontinue metformin at the time of or prior to iodinated contrast imaging pr
ocedures in patients with eGFR between 30 and 60 mL/min/1.73 m2.

4 CONTRAINDICATIONS

- Severe renal impairment (eGFR below 30 mL/min/1.73 m2).
- Known hypersensitivity to metformin hydrochloride.
- Acute or chronic metabolic acidosis, including diabetic ketoacidosis, with or
without coma.

5 WARNINGS AND PRECAUTIONS

- Lactic Acidosis: See Boxed Warning.
- Vitamin B12 Deficiency: Metformin may decrease vitamin B12 levels. Measure hem
atologic parameters annually and B12 at 2- to 3-year intervals.
- Hypoglycemia: Does not occur in patients receiving metformin alone under usual
 circumstances, but could occur in caloric deficiency, strenuous exercise, or co
mbined with sulfonylureas or insulin.

6 ADVERSE REACTIONS

Most common adverse reactions (>5%): Diarrhea, nausea, vomiting, flatulence, ast
henia, indigestion, abdominal discomfort, headache, and metallic taste in mouth.
 Adverse gastrointestinal events are most common during initiation and often tra
nsient.

7 DRUG INTERACTIONS

- Carbonic Anhydrase Inhibitors (e.g., Topiramate, Zonisamide): May increase ris